Bevacizumab 15 mg/kg · lung treatment component

Treatment scope · NCCP 00215 Lung context only. Confirm whether the platinum pathway or EGFR/erlotinib pathway applies. This guide does not supply the partner dose or replace its safety assessment.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Bevacizumab lung component · support plan

Active medicines: Bevacizumab

Emetogenic risk: Minimal for bevacizumab only

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Bevacizumab itself usually needs no routine anti-sickness premedication. Platinum chemotherapy needs its own anti-sickness and other support plan. Erlotinib needs its own tablet, interaction, skin and diarrhoea plan. Keep the complete plan and emergency numbers.
- No routine bevacizumab antiemetic prophylaxis; assess partner chemotherapy emetic risk separately.
- Anti-diarrhoeal medicines only if required under local policy.
- Maintain BP/urine, procedure/wound and urgent-contact plans.
- At the platinum-to-bevacizumab continuation change, review and stop or adapt partner-specific support only on the prescriber’s instruction. Erlotinib support continues according to its own plan.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00215 v7 · p6 · Current NCCP regimen supportive-care section

Sources checked 2026-10-29 · SACTCheck v0.84.25 · Regimen-level drafts; prescriber and oncology-pharmacy verification required